Exam Case 2

CANDIDATE'S INSTRUCTIONS
A woman approaches you regarding a concern. Please counsel and address any concerns as you would in practice.

Time Frame: You have 7 minutes to complete this station

References: CTMA chapter constipation in children, PEG product monograph

On the table:
Lactulose, Glycerin suppositories, Magnesium hydroxide (MOM), PEG (e.g. Lax-a- day), Senna

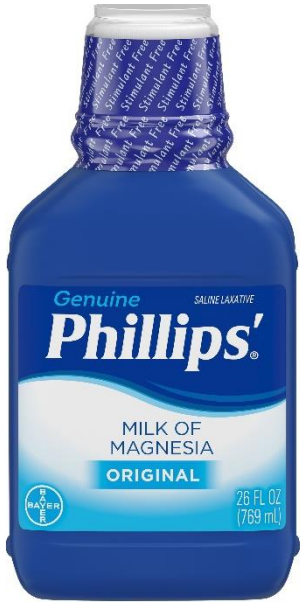
TIPS
PharmacyPREP.COM

Lactulose

PEG

Glycerin suppositories

Senna



Milk of Magnesia

Standardized patient:

Opening statement: What's the best medication for constipation?

If a candidate asks: Did the child see a doctor? → Answer: No, **does he need to?**

If candidate asks: Has he tried anything yet? → Answer: Yes, tried increasing vegetables but he doesn't like them. Also tried glycerin suppositories in the past but he didn't like them because of discomfort (screaming and kicking).

5-minute question: I have **mineral oil** at home; can I give him that?

Patient/Child information: **Mike**

Mother's name: Maria

- **3 years old, 15 kg**, male, no allergies, no medications
- No bowel movement in the **past 3 days.**
- Not the first time, chronic constipation for the past several weeks since toilet training.
- Small, hard stools, difficult to pass
- Moderate discomfort – pain on defecation which causes fecal retention → does not like toileting
- No signs of impaction (rectal discomfort, nausea/vomiting, abdominal pain…)
- No red flags (fever, nausea, vomiting, abdominal pain)
- **Diet: does not like vegetables, does not like to drink water**
- Wants medication for treatment

Problem solved

- Check for red flags, impaction/ bowel obstruction
- Patient education: “normal” BM frequency varies, non-pharms, develop bowel routine, monitoring
- Non-pharms: create routine for toileting e.g. attempt 5-15 min before every meal until 1 BM; dietary fibre (age plus 5g/day; or 10 g/day); sorbitol/barley malt/corn syrup (stool softener); prune; diet (fruits, vegetables, whole grains); exercise and hydration
- Pharms:
  - Best: PEG : is the primary choice for most children 0.4-0.8 g/kg/d in 240 ml fluid (max 17g) it often takes **24–48 hours** for bowel evacuation to be achieved.
  - **Lactulose is considered a second-line agent and generally used only if PEG 3350 is unavailable.**
  - lactulose 5 ml bid [or sorbitol 1-3ml/kg/d in divided doses] (osmotic agents) → **onset 24-48h for both**

  Others

  Second line: senna, stool softeners (e.g. docusate)
  - Glycerin suppositories: previous bad experience (traumatizing). Also, it is less effective with dry, hard stools.
  - Magnesium hydroxide: second line (SE), the patient doesn’t like to drink water (risk of dehydration, electrolyte disturbance)
  - Avoid mineral oil – risk of lipid aspiration and pneumonia (especially in children)

  Provide complete counselling (clear instructions,(present in reference) onset/expectation, S/Es, when to see doctor)
- Offer to monitor (with timeframe), follow up
  - see MD if >2 weeks chronic constipation or no BM > 7 days despite laxatives; **acute constipation not relieved in 2-3 days**; chronic constipation: 4-week trial to achieve regular pattern; SE – 3 days (bloating, cramping should be relieved by 3 days)
  - Refer if blood or mucus in stool or rectal bleeding, fever, no weight gain or weight loss, persistent abdominal pain, vomiting, diarrhea alternating with constipation (potential IBS), rectal/abdominal mass
- Refer: CTMA: Chapter constipation in children, PEG product monograph.

Marginally solved

Check for red flags, impaction/ bowel obstruction
Patient education: “normal” BM frequency varies, non-pharms, develop bowel routine, monitoring
Non-pharms: create routine for toileting e.g. attempt 5-15 min before every meal until 1 BM; dietary fibre (age plus 5g/day; or 10 g/day); sorbitol/barley malt/corn syrup (stool softener); prune; diet (fruits, vegetables, whole grains); exercise and hydration
Pharms:
Best: PEG : is the primary choice for most children 0.4-0.8 g/kg/d in 240 ml fluid (max 17g) it often takes **24–48 hours** for bowel evacuation to be achieved.
Lactulose is considered a second-line agent and generally used only if PEG 3350 is unavailable.
Lactulose 5 ml bid [or sorbitol 1-3ml/kg/d in divided doses] (osmotic agents) → **onset 24-48h for both**
Did not address 5th min. question.
Follow-up didn’t mention.

Marginally unsolved/unsolved

Check for red flags, impaction/ bowel obstruction
Patient education: “normal” BM frequency varies, non-pharms, develop bowel routine, monitoring
Non-pharms: create routine for toileting e.g. attempt 5-15 min before every meal until 1 BM; dietary fibre (age plus 5g/day; or 10 g/day); sorbitol/barley malt/corn syrup (stool softener); prune; diet (fruits, vegetables, whole grains); exercise and hydration
Do not recommend product or recommend other products.
Did not address 5th min. question.
Follow-up didn’t mention.

Pharmacy PREP OSCE Check List Case LACTULOSE

Case Title. Infant constipated

Communication skills (checklist)	**Outstanding**	**Good**	Acceptable	**NOT Acceptable**
Opening Statement (respond to patient clue, good listener, did not interrupt patient, engaging). Non-verbal Skills (eye contact, gesture, posture), Identified patient?	**4**	**3**	**2**	1
Information gathering process. (Asked M-A-M-A) allergies, medicine history, medical conditions. Builds trust and confidence with patient, Specific lifestyle such as alcohol and smoking habits prn; and/or other considerations relevant to the purpose of the case.	**4**	**3**	**2**	1
Discuss Follow up/Monitoring relevant to case	**4**	**3**	**2**	1
Time to check references, and use of proper references	**4**	**3**	**2**	1
Overall time management	**4**			
Overall communications	**Outstanding (20)**	**Good (15-20)**	Acceptable (12-14)	**(<14) NOT Acceptable**

Problem Solving (checklist)	Fully solved (FS)	Marginally solved (MS)	Marginally unsolved (MU)	Unsolved (U)
key elements of the counseling task Check for red flags, impaction/ bowel obstruction Patient education: "normal" BM frequency varies, non pharms, develop bowel routine, monitoring Nonpharms: create routine for toileting e.g. attempt 5-15 min before every meal until 1 BM; dietary fibre (age plus 5g/day; or 10 g/day); sorbitol/barley malt/corn syrup (stool softener); prune; diet (fruits, vegetables, whole grains); exercise and hydration Pharms: Best: lactulose 5 ml bid [or sorbitol 1-3ml/kg/d in divided doses] (osmotic agents) → onset 24-48h for both Others (not best choice): Second line: senna, stool softeners (e.g. docusate) see MD if >2 weeks chronic constipation or no BM > 7 days despite laxatives;	All key points covered	3 key points were covered from 4 point	2 key points were covered from 4 point	Improper Recommendation

Misinformation?	**O**	**Put patients in to risk?**		**O**

Overall Performance	**Superior(O+FS)**	**Good(G/A +FS)**	**Fair(A +MS/MU)**	**Need to improve(U+MU/U)**